

IN ASSOCIATION WITH APOLLO HOSPITALS · FINANCIAL
DISTRICT, HYDERABAD

DRAFT – SCIENTIFIC COMMITTEE

HSC 2027 · INAUGURAL EDITION

RETURN TO SPORT PROGRAM PLAN

HYDERABAD SPORTS INJURIES CONCLAVE

30TH & 31ST JANUARY 2027

Prepared for the Scientific Committee

Researched against IASCON · IASMCN · ISKSAA · ISAKOS · AOSSM · ESSKA · IFSPT · NSCA
and the IOC, WADA and ACSM consensus literature

Organised by Precision Orthopaedics LLP · precisionortho4488@gmail.com

ONE THEME, TWO HALLS

Return to sport is the theme of HSC 2027. Every session answers one question from a different angle — how do we get this athlete back, safely and at full performance? Two parallel halls give the surgical and the rehabilitation audiences a full two days of depth each, while sports science and nutrition are woven into the sessions where they belong rather than separated into a track of their own. Joint plenary sessions keep it one conclave.

HALL A

Surgery & Sports Orthopaedics

Orthopaedic & arthroscopy surgeons, sports physicians, PG trainees · ~55% of delegates

HALL B

Rehabilitation, Physiotherapy & Strength and Conditioning

Sports physiotherapists, S&C coaches, athletic trainers, sports scientists · ~45%

Room sizing: Hall A doubles as the plenary hall and must seat the full delegate count. Hall B needs roughly half that, plus clear floor space for the hands-on workshops.

PLENARY SESSIONS — EVERYONE IN ONE ROOM

(Called "plenary," not "joint" — this document is organised by anatomical joint throughout, and reusing that word for combined-hall sessions was its own source of confusion.) These sessions are deliberately **not** parallel — and they are where the theme lives:

SESSION	WHEN	WHY IT MUST BE PLENARY
Inauguration	Day 1, 09:00	Ceremonial; sets the inaugural-edition tone
Opening keynote	Day 1, 09:15	One marquee name, heard by every delegate
Return to Sport: The Case, Together surgeon → physiotherapist → S&C coach → floor debate	Day 1, 12:45	The signature session. Run once, for the whole conclave, to the exact structure Hall A then splits into nine parallel cases — see the box on page 4
Medical ethics & good clinical practice	Day 1, 17:30	Mandatory for TSMC CPD accreditation — the Declaration Proforma requires it. Plenary scheduling guarantees attendance capture
Concussion — consensus-based practice	Day 2, 12:05	Sideline removal and graded return concern surgeons, physiotherapists and coaches equally — it belongs to both halls
Athlete fireside chat	Day 2, 12:35	Draws every audience; strongest media moment
Awards & valedictory	Day 2, 17:00	Best paper and poster across both tracks

Where the sports-science content went. Nutrition, energy availability, psychology and load management are not a separate hall — they sit inside Hall B, with the people who apply them: *Fuelling recovery* and load management beside injury risk, and concussion raised to a plenary. Anti-doping and sports cardiology run as a Hall B session during Hall A's Day 2 closing tea break (16:30) — Hall A's nine case blocks fill every available minute with no slack for a tenth, so this is the one science block that could not be promoted to plenary this edition. Para-athlete health and the youth athlete remain reserve blocks or a pre-conference day.

WHAT EACH ONE ACTUALLY COVERS

The grid on page 3 only carries a title. Here is what a delegate is walking into for every plenary except Return to Sport: The Case, Together, whose full running order is on page 4.

Inauguration · 09:00, ~15 min

Formal opening — welcome address by the Chairman, brief remarks from Precision Orthopaedics LLP and Apollo Hospitals, and the Return to Sport theme framed for the two days ahead. Ceremonial, not content-bearing — the reason it can't move is protocol, not programme.

Opening keynote · 09:15, ~45 min

One marquee name, one talk, a career-spanning view of return to sport rather than a technique deep-dive — the tone-setter for everything that follows. Speaker TBC; this is the single highest-leverage name to confirm first, since it's heard by every delegate in the building.

Medical ethics & good clinical practice · 17:30, ~45 min

The TSMC-mandated session. Should cover informed consent in elective sports surgery, managing conflicts of interest with equipment and pharma sponsors — directly relevant given the orthobiologics content in Hall A and the equipment-sponsorship boundary in Section 7 — and documentation standards. Attendance is captured via the Declaration Proforma, which is why this has to be plenary rather than parallel.

Concussion — consensus-based practice · Day 2, 12:05, ~30 min

Built on the Amsterdam 2022 (6th International) consensus statement: the new visible signs mandating immediate removal from play, SCAT6/SCoAT6 in practice, and a graded return-to-play protocol realistic for Indian school and club sport without on-site physician cover.

Athlete fireside chat · Day 2, 12:35, ~35 min

A moderated conversation with a competitive athlete about their own injury and return journey — the patient's side of everything both halls have spent two days debating. Athlete TBC; the session most likely to get picked up by local press, so worth securing early.

Awards & valedictory · Day 2, 17:00, ~45 min

Best paper and best poster, one prize per track (surgical / rehab / science, per Section 7's decision on prize categories) so entries don't compete across disciplines. Chairman's closing remarks, formal close of the inaugural edition.

DAY 1 — SATURDAY 30 JANUARY 2027

TIME	HALL A — CASE-BASED DEBATE		HALL B — REHABILITATION, PHYSIOTHERAPY & S&C
08:00	Registration · breakfast · trade exhibition		
09:00	PLENARY — Inauguration		
09:15	PLENARY — Opening keynote: <i>Return to sport — the decision that defines us</i>		
10:00	CB1 · Recurrent shoulder instability in the thrower	FLAGSHIP CASE	Criteria-based rehabilitation & the return-to-play decision
10:00	Chair Presents the case		
10:02	Surgeon 1 Bankart + remplissage		
10:12	Surgeon 2 Latarjet — the bone-loss threshold		
10:22	Surgeon 3 Free bone block, or the case against operating in-season		
10:30	Physio Return-to-throwing progression		
10:40	S&C Shoulder loading & scapular control		
10:48	Floor (20m) The case, together		
11:08	Tea · exhibition		
11:23	CB2 · ACL rupture in the pivoting athlete		Objective testing & return-to-sport criteria
11:23	Chair Presents the case		
11:25	Surgeon 1 Hamstring graft		
11:35	Surgeon 2 Quadriceps tendon graft		
11:45	Surgeon 3 Repair + internal bracing, or add an LET		
11:53	Physio Criteria-based RTP, not a calendar date		
12:03	S&C Kabaddi-specific loading pattern		
12:11	Floor (30m) The case, together		
12:41	PLENARY — RETURN TO SPORT: THE CASE, TOGETHER · surgeon → physiotherapist → S&C coach → floor debate		
13:26	Lunch · exhibition		
14:26	CB3 · Rotator cuff in the overhead athlete		Tendinopathy & load-driven pain
14:26	Chair Presents the case		
14:28	Surgeon 1 Conservative care, properly done, first		
14:38	Surgeon 2 Arthroscopic debridement — no formal repair		
14:48	Surgeon 3 Arthroscopic repair — transtendon preferred		
14:56	Physio GIRD/posterior-capsule work, criteria-based return to serve loading		
15:06	S&C Core-kinetic-chain deficit & velocity-based return		
15:14	Floor (20m) The case, together — tournament six weeks out: operate now, or push conservative care through it?		
15:34	CB4 · The torn meniscus — save it or not		Fuelling recovery — energy availability, REDs & nutrition periodisation
15:34	Chair Presents the case		
15:36	Surgeon 1 Repair		
15:46	Surgeon 2 Partial meniscectomy		
15:56	Surgeon 3 "Meniscectomy is not a crime" — where resection is right		
16:04	Physio Protected weight-bearing vs accelerated return		
16:14	S&C Long-term load management		
16:22	Floor (20m) The case, together		
16:42	Tea · exhibition		
17:32	PLENARY — Medical ethics & GCP in sports medicine (<i>TSMC CPD requirement</i>)		
18:17	Free papers — all podium free papers, both halls (~14 talks, 8+2 min)		
19:32	Faculty dinner		

DAY 2 — SUNDAY 31 JANUARY 2027

TIME	HALL A — CASE-BASED DEBATE	HALL B — REHABILITATION, PHYSIOTHERAPY & S&C
08:30	CB5 · Hamstring tear & reconditioning 08:30 Chair Presents the case 08:32 Surgeon/physician 1 Conservative management 08:42 Surgeon/physician 2 Surgical repair threshold 08:52 Surgeon/physician 3 What MRI actually changes about Monday 09:00 Physio L-protocol vs conventional timelines 09:10 S&C Sprint/hurdle reintroduction dosing 09:18 Floor (20m) The case, together	W2 · Field-based RTP testing & hamstring (hands-on)
09:38	CB6 · The unstable ankle & stress fractures around it 09:38 Chair Presents the case 09:40 Surgeon 1 Modified Bröstrom 09:50 Surgeon 2 Augmented reconstruction 10:00 Surgeon 3 When the stress fracture changes the plan 10:08 Physio Proprioceptive rehab vs long-term bracing 10:18 S&C Load progression back to cutting sports 10:26 Floor (20m) The case, together	Muscle injury & the hamstring
10:46	Tea · exhibition	
11:01	CB7 · Hip & groin pain in the athlete 11:01 Chair Presents the case 11:03 Surgeon 1 Arthroscopy threshold after physio failure 11:13 Surgeon 2 Labral repair vs reconstruction 11:23 Surgeon 3 Dysplasia — scope + plication vs PAO 11:31 Physio Groin-pain clinical-entity classification 11:41 S&C The loading programme that should have come first 11:49 Floor (30m) The case, together	Load management & injury risk
12:19	PLENARY — Concussion — consensus-based practice	
12:49	PLENARY — Athlete fireside chat	
13:24	Lunch · exhibition	
14:24	CB8 · Fast bowler's lumbar stress fracture CRICKET MARQUEE 14:24 Chair Presents the case 14:26 Surgeon/physician 1 Conservative rest-and-graduated-return 14:36 Surgeon/physician 2 Imaging thresholds for return 14:46 Surgeon/physician 3 The rare surgical indication 14:54 Physio Graduated bowling-load return protocol 15:04 S&C Which workload metric actually predicts this 15:12 Floor (30m) The case, together	The athlete who isn't a 25-year-old male
15:42	CB9 · Hand & wrist fractures in sport 15:42 Chair Presents the case 15:44 Surgeon 1 Cast — distal-pole/incomplete 15:54 Surgeon 2 Percutaneous screw — nondisplaced waist, RTP speed 16:04 Surgeon 3 ORIF/dual-screw — proximal-pole/unstable 16:12 Physio Early mobilisation vs immobilisation 16:22 S&C Grip/loading return 16:30 Floor (30m) The case, together	Young Surgeon Forum Final — case vs case knockout (page 4f)
17:00	Tea	
17:30	PLENARY — Awards & valedictory	
	Anti-doping & sports cardiology — WADA 2026 updates	

Reserve blocks (add with a pre-conference day, or swap in as faculty confirm): CB10 elbow OCD in the overhead athlete/gymnast · CB11 acute traumatic full-thickness rotator cuff tear · CB12 SLAP tear in the overhead athlete · CB13 acute ATFL repair — the first-time high-grade ankle sprain · CB14 distal biceps tendon rupture · CB15 revision ACL reconstruction — the case that came back · CB16 Achilles tendon rupture · CB17 patellofemoral instability · CB18 multiligament knee injury / knee dislocation · CB19 syndesmosis · CB20 subtle Lisfranc · CB21 knee cartilage defect · CB22 grade III AC joint · adjuncts under

CASE-BASED DEBATE

Format — one real patient per block, not a survey of sub-topics. Chair presents an actual case, 2 min. Three surgeons give genuinely different views — 10 + 10 + 8 min, hard stops enforced by chairs. Physiotherapist, 10 min: the rehab arc for whichever path is chosen. S&C coach, 8 min: return to performance, not just return to play. Then **20–30 protected minutes** (30 for the four blocks with the deepest three-way surgical splits — CB2, CB7, CB8, CB9; a firm 20 for the rest) — the floor puts the same case back to the whole panel: "he's in front of you now — what do you actually do?" Audience polls before and after. 68–78 minutes total; this is the Chairman's own design, and no published conference anywhere runs the exact same format — the closest precedents are AOSSM's case-based Instructional Course and ESSKA's Hot Topic Debate, both narrower. **Pre-record all "live" surgery** shown during a surgeon's slot — the operating surgeon narrates the recording live, which gives the teaching value without OT-link risk.

CB1 · Recurrent shoulder instability in the thrower/bowler FLAGSHIP — OPENS DAY 1

The case: 19-year-old fast bowler, first anterior dislocation diving to field a shot 18 months ago, two further recurrences since; glenoid bone loss on 3D-CT (attributable to the fielding-collision trauma, not the bowling action itself, which drives GIRD/internal impingement instead). **Surgeon 1** argues arthroscopic Bankart + remplissage; **Surgeon 2** argues Latarjet, and where the bone-loss threshold actually sits; **Surgeon 3** makes the case for a free bone block, or argues against operating at all in-season. **Physiotherapist:** return-to-throwing progression — not return-to-sport in general, the throwing motion specifically. **S&C coach:** shoulder loading programme, scapular control, when full-velocity bowling is reintroduced. This is the case the organisers described first — every other block in Hall A follows its structure.

THE FLOOR, TOGETHER · he's a state-level bowler, 3 dislocations in 18 months, season starts in 4 months — what do you actually do?

Invited (subject to confirmation, final 3 surgeons TBC): Dr Deepak N. Bhatia · Dr Sujit Jos · Dr Dinshaw Pardiwala · Prof Teruhisa Mihata · Dr Denny T. T. Lie · Giovanni Di Giacomo · Emilio Calvo · Clara Azevedo · Dr Eric McCarty · Dr Jonathan Dickens · Dr Stephen Brockmeier · Dr Brian Wolf · Dr Robert Arciero · Dr Nikhil Verma · Dr Matthew Provencher · Physio: Nitin Patel · S&C: Ramji Srinivasan

CB2 · ACL rupture in the pivoting athlete

The case: 23-year-old female kabaddi or football player, complete ACL tear, meniscal involvement — female specifically, since ACL injury risk runs roughly 1.5–3× higher in female athletes. **Surgeons** debate graft choice (hamstring vs quadriceps tendon vs BTB), repair-with-augmentation for the right 5%, and lateral extra-articular tenodesis or not. **Physiotherapist:** criteria-based RTP — not a 9-month calendar date. **S&C coach:** the kabaddi-specific loading pattern; India's own data (89.5% ACL tear rate in injured kabaddi players) makes this the most locally relevant case in the programme.

Invited (subject to confirmation): Dr Raju Easwaran · Dr Ish Middha · Mr Sanjeev Anand · Physio & S&C: TBC from the invited pool

CB3 · Rotator cuff in the overhead athlete

The case: 27-year-old national-circuit competitive tennis player (singles), 8-month history of deep posterosuperior shoulder pain in the late-cocking/deceleration phase of the serve; GIRD ~25° vs the non-dominant side, scapular dyskinesis, intact anterior stability and an intact biceps-labral complex (not the SLAP-dominant case CB12 covers). Four months of generic (non-internal-impingement-specific) physiotherapy gave partial relief. MR arthrogram: Ellman grade III (>50% thickness) articular-sided partial-thickness supraspinatus tear — a PASTA lesion — with preserved muscle bulk and no fatty infiltration. **Surgeons** debate a properly-run internal-impingement-specific conservative programme first (the prior physio trial wasn't the right one) vs arthroscopic debridement without formal repair (still the commonly taught default for these lesions) vs arthroscopic repair, transtendon technique preferred (2025 evidence shows debridement alone underperforms repair even at this grade; genuine equipoise remains between transtendon and tear-completion-and-repair technique once repair is chosen). **Physiotherapist:** GIRD-directed posterior-capsule stretching and scapular/kinetic-chain retraining, criteria- not calendar-based return to serve-specific loading. **S&C coach:** the core-endurance and scapular-stabiliser deficit that drove the overload in the first place, velocity-based return-to-serve progression.

Invited (subject to confirmation): Chair TBC · Physio & S&C: TBC from the invited pool

CB4 · The torn meniscus — save it or not

The case: posterior root tear with extrusion in a 56-year-old recreational athlete, varus alignment — a degenerative medial-root presentation, not the younger traumatic/ACL-associated lateral-root pattern. **Surgeons** debate repair vs partial meniscectomy vs "meniscectomy is not a crime" — the cases where resection is genuinely right. **Physiotherapist:** post-repair protected weight-bearing vs post-meniscectomy accelerated return. **S&C coach:** the load-management case for protecting a repaired meniscus long-term.

Invited (subject to confirmation): Dr Naman Goel · second/third surgeon & physio/S&C: TBC from the invited pool

CB5 · Hamstring tear & reconditioning

The case: 26-year-old track-and-field hurdler, proximal hamstring tendon avulsion — 2 of 3 tendons, >2cm retraction, clearing a hurdle — failed an initial conservative trial. **Surgeons/sports physicians** debate conservative vs surgical repair thresholds at this retraction/tendon-count level, and what MRI actually changes about Monday's decision. **Physiotherapist:** L-protocol vs conventional rehab timelines, lengthened-state training. **S&C coach:** sprint/hurdle-clearance reintroduction dosing — the reinjury-risk case for not rushing return-to-hurdling.

Invited (subject to confirmation): Chair — Dr Vivekanand Upadhyay / Dr Bhupesh Karthik / Dr Aditya Sai · Physio: Tracy Fernandes · S&C: Naresh Ramadoss

CB6 · The unstable ankle & stress fractures around it

The case: 27-year-old elite basketball player, multi-year sprain history, chronic lateral ankle instability presenting alongside a missed navicular stress fracture — basketball specifically, since navicular stress fracture is the single most common site requiring surgery among lower-extremity stress fractures in elite basketball cohorts. **Surgeons** debate modified Broström vs augmented reconstruction, and when the stress fracture changes the surgical plan entirely. **Physiotherapist:** proprioceptive rehab vs long-term bracing. **S&C coach:** load progression back to cutting sports — why the stress fracture happened.

Invited (subject to confirmation): Dr Debasis Chatterjee · second/third surgeon & physio/S&C: TBC from the invited pool

CB7 · Hip & groin pain in the athlete

The case: 28-year-old footballer with FAI, labral pathology and a concurrent adductor-related component, failed a physiotherapy trial. **Surgeons** debate the intra-articular component only — arthroscopy threshold after physio failure, labral repair vs reconstruction, and borderline dysplasia — scope or PAO; the adductor pain is managed in parallel via an adductor-loading programme, not resolved by any of these three positions. **Physiotherapist:** the groin-pain clinical-entity classification and where physio genuinely changes the outcome. **S&C coach:** the loading programme that should have been tried before referral.

Invited (subject to confirmation): Chair — Dr Vivekanand Upadhyay / Dr Bhupesh Karthik / Dr Aditya Sai · Physio: Patrick Farhart · S&C: TBC from the invited pool

CB8 · Fast bowler's lumbar stress fracture CRICKET MARQUEE — PRIME DAY 2 SLOT

The case: 17-year-old academy fast bowler, contralateral pars stress reaction on MRI. **Surgeon/sports physician** debate conservative rest-and-graduated-return vs the rare surgical indication, and imaging thresholds for return. **Physiotherapist:** the graduated bowling-load return protocol. **S&C coach:** bowling-workload monitoring — which metric actually predicts this injury and which don't. The single most India-relevant case in the programme; record it and clip it for the brochure.

Invited (subject to confirmation): Surgeon/sports physician seat OPEN — no verified candidate after two research passes, sourcing through the committee's own network · Physio: Nitin Patel or Andrew Leipus · S&C: Ramji Srinivasan or Basu Shanker

CB9 · Hand & wrist fractures in sport

The case: 22-year-old cricketer or kabaddi player, scaphoid fracture from a fielding dive or tackle onto an outstretched hand, plus a brief second case — a fielded-ball direct-blow metacarpal fracture or a catching-attempt PIP joint dislocation — as a rapid-fire coda. **Surgeons** debate cast for a distal-pole/incomplete fracture vs percutaneous screw for a nondisplaced waist fracture (arguing return-to-play speed alone justifies surgery) vs ORIF/dual-screw for a proximal-pole/unstable pattern. **Physiotherapist:** early mobilisation vs immobilisation. **S&C coach:** grip/loading return and the decision on competing before full union. Closes Hall A's case series.

Invited (subject to confirmation): Dr Debasis Chatterjee (possible dual booking with CB6) · second/third surgeon & physio/S&C: TBC from the invited pool

Format sources: AOSSM Instructional Course guidelines (Case-Based format spec) · ESSKA session types (Hot Topic Debate spec) · AOSSM 2025 "You Can Take That to the Bank-Art" full debate programme · ISAKOS Regional Summit Mumbai 2026 (the closest India-hosted precedent). **Clinical sources:** IASCON 2025 & 2026 programmes · ISAKOS 2025 Congress · ESSKA 2026 · ISKSAA · LET international consensus (Arthroscopy 2025) · quadriceps tendon global survey (JEO 2026) · ramp/root/radial current concepts (JASSM 2025) · ESSKA-ORBIT PRP consensus · PAO vs hip arthroscopy meta-analysis 2025 · kabaddi knee injury epidemiology (Asian J Sports Med). Named faculty candidates for every block, with sources, are held separately in the committee's internal outreach document.

RESERVE BLOCKS – CB10–CB14

Same template as CB1–CB9 (chair 2 min · three surgeons 10+10+8 · physio 10 · S&C 8 · floor 20–30). Not on the two-day grid — add with a pre-conference day, or swap in for a current block once faculty confirm. No faculty outreach has started on any reserve block; that is deliberately sequenced after the committee decides which are wanted. **Three further reserves (CB16–CB18) from the August 2026 slate-level review are on page 4c; CB15 and CB19–CB22, from the September 2026 benchmark against “Back in the Game”, are on pages 4d–4e.**

CB10 · Elbow OCD in the overhead athlete/gymnast

The case: 13-year-old competitive gymnast, 4 months of activity-related lateral elbow pain, intermittent catching, 15° terminal extension deficit; MRI shows a 12mm capitellar OCD lesion with a complete peripheral fluid line (unstable), open physes. **Surgeon 1** argues conservative rest/bracing even against an unstable lesion — growth remaining and an intact cartilage cap still favour biological healing; **Surgeon 2** argues arthroscopic debridement/microfracture — the lesion is unstable enough to act on now, and marrow stimulation is a lower-morbidity first surgical step; **Surgeon 3** argues osteochondral autograft transplantation (OATS) — once a lesion clears the ≥10mm/unstable threshold, as this one does, anatomic restoration is preferred. **Physiotherapist:** kinetic-chain and valgus-loading mechanics, staged ROM — no RTS consensus exists yet, most protocols are still time-based (~6 months) rather than functional. **S&C coach:** youth-specific load management, pitch-count/gymnastics-load caps around peak-height-velocity windows, the early-specialisation argument. Fills the long-standing “elbow & overhead athlete” reserve slot with a concrete case.

CB11 · Acute traumatic full-thickness rotator cuff tear

The case: 58-year-old active patient, anterior shoulder dislocation from a fall, delayed MRI at 10 weeks shows a massive full-thickness supraspinatus + infraspinatus tear, stage 1–2 fatty infiltration, Hamada grade 1. **Surgeon 1** argues primary repair despite the delay — traumatic tears behave differently from degenerative ones, and delayed repair hasn't been shown to worsen outcomes; **Surgeon 2** argues joint-preserving reconstruction (SCR or balloon spacer) — the more cost-effective strategy in most massive-irreparable-tear models; **Surgeon 3** argues primary reverse TSA in the active-but-aging patient — the definitive fix now, not a joint-preservation detour. **Physiotherapist:** three genuinely different rehab arcs depending on the path chosen. **S&C coach:** the loading ceiling forks sharply — full return to overhead/manual work after repair vs a permanent overhead-lifting restriction after RSA. Distinct from CB3's atraumatic, partial-thickness overhead-athlete case — same joint, different patient and different debate.

CB12 · SLAP tear in the overhead athlete

The case: 19-year-old Division-I-calibre pitcher, deep posterior shoulder pain in late cocking/acceleration, measurable velocity drop, positive O'Brien's and biceps-load-II, MR arthrogram confirms a type II SLAP lesion with an intact cuff — explicitly negative for instability. **Surgeon 1** argues arthroscopic SLAP repair — the traditional position, conceding 40–80% pitcher RTP and a 7.5–12.5% revision rate; **Surgeon 2** argues biceps tenodesis as the index procedure — gaining real ground even in throwers under 35, non-inferior on outcomes; **Surgeon 3** argues conservative, rehab-first — ~40% of pro pitchers return on rehab alone. **Physiotherapist:** anchor-protective phase before resisted supination if repair is chosen, earlier and more aggressive biceps loading if tenodesis is chosen. **S&C coach:** scapular control, hip-trunk sequencing, velocity-based return-to-throwing testing. Distinct from CB1 (frank instability, positive apprehension) — schedule away from it in the running order so the “instability vs pain-generator” distinction registers.

CB13 · Acute ATFL repair — the first-time high-grade ankle sprain

The case: 24-year-old professional field-sport athlete, inversion injury 10 days before a season-defining tournament, complete mid-substance ATFL tear on MRI, intact CFL, no prior instability. **Surgeon 1** argues functional rehab first — most first-time sprains, even high-grade, do well without surgery; **Surgeon 2** argues acute anatomic repair without augmentation — restores native anatomy, augmentation over-treats a repairable tear; **Surgeon 3** argues acute repair + internal-brace/suture-tape augmentation — superior load-to-failure, a meaningfully accelerated protected-loading timeline, increasingly favoured at elite level. **Physiotherapist:** the rehab arc diverges sharply by pathway. **S&C coach:** return-to-cutting/pivoting progression compressed for the augmented pathway, standard otherwise, watching for reinjury risk from schedule-driven acceleration. Distinct from CB6 (chronic/recurrent instability, a missed stress fracture) — this is the acute, first-event decision.

CB14 · Distal biceps tendon rupture

The case: 32-year-old competitive powerlifter, felt a pop with sharp antecubital pain during a max-effort deadlift, "Popeye" deformity, positive hook test, MRI confirms complete rupture with up to 2cm retraction, presents day 12 — still a clean window for direct primary repair. Nonoperative management isn't a credible position here — meaningful supination-strength deficits persist without repair — so all three surgeons operate and disagree only on technique/timing: **Surgeon 1** single-incision cortical button — best strength/endurance data, delay past 4 weeks risks needing graft; **Surgeon 2** double-incision transosseous fixation — historic gold standard, lower PIN-injury risk, trades for H0/synostosis risk and a second scar; **Surgeon 3** suture-anchor fixation — biomechanically comparable to a button, easier revision if it fails. **Physiotherapist:** extension-block splinting the first 2 weeks, then criteria-based progression to axial loads. **S&C coach:** early isometric grip/shoulder work, blood-flow-restriction training, staged return-to-pull criteria — realistic timeline 4–6 months to heavy pulling, 9–12 to compete. Genuinely new territory, nothing like it on the current grid. Distal triceps rupture (the powerlifter/bodybuilder variant, linked to anabolic steroid use) is real but thinner — a one-line floor-debate coda here, not its own block for now. **Second coda (Sep 2026):** pectoralis major rupture was researched and rejected as its own block (overlaps this one, and non-operative care in athletes is settled) — ask the floor: "would you still operate on a chronic, retracted pec major tear in a steroid-using bodybuilder?"

Sources: PASTA repair vs debridement (JSES Rev Rep Tech 2025) · traumatic vs degenerative cuff tear repair outcomes (JSES Int 2022; Int Orthop 2024) · SCR cost-utility modelling (J Exp Orthop 2025) · SLAP repair vs biceps tenodesis meta-analysis (AJSM 2021) and decision analysis (Arthroscopy 2022) · tenodesis fixation-site RCT/registry data (AJSM 2022; ASMR 2023) · ATFL suture-tape augmentation biomechanics (Orthopaedic Surgery 2024) · ISAKOS ligament-augmentation survey (J ISAKOS 2023) · capitellar OCD OATS indications (JAAOS 2025) · elbow OCD return-to-sport review (JSES Int 2023) · distal biceps repair technique outcomes (JSES Rev Rep Tech 2026; Cureus 2025) · distal triceps repair and anabolic steroid risk (Hand (N Y) 2022). Full citations with DOIs held in the committee's internal research notes.

RESERVE BLOCKS – CB16–CB18

Added after the August 2026 slate-level review, which examined the topic list as a whole rather than block by block. It found two outright holes (Achilles rupture, patellofemoral instability), one India-specific gap (the multiligament knee), an empty 30–55 age band across the scheduled nine, and – before CB2 and CB17 – not a single female athlete in fourteen cases. These three blocks close all of it. Same template as CB1–CB9.

CB16 · Achilles tendon rupture

The case: 34-year-old competitive club badminton player, push-off for a backhand lunge, audible pop and the classic “felt kicked from behind,” presents day 6. Palpable 4cm gap, positive Thompson test, resting plantarflexion asymmetry; ultrasound shows full-thickness rupture 4cm from the insertion, 12mm gap in equinus. **Surgeon 1** argues non-operative management with accelerated functional rehab – identical 12-month function, and it avoids infection and nerve injury entirely; **Surgeon 2** argues open repair – roughly ten-fold re-rupture reduction, and an athlete who must push off cannot absorb a ~6% career-ending risk; **Surgeon 3** argues percutaneous/minimally-invasive repair – re-rupture equal to open, infection near-zero, priced in sural nerve palsy. The 2022 *NEJM* three-arm trial (554 patients) is the spine of the block: no difference in patient-reported outcome across all three arms, but re-rupture 6.2% / 0.6% / 0.6% and 5.2% nerve injury in the MIS arm. **Physiotherapist:** functional bracing, heel-wedge weaning, tendon elongation monitored via heel-rise height – and the honest finding that accelerated vs delayed rehab showed no outcome difference, so what actually drives recovery? **S&C coach:** the plantarflexor deficit that outlasts every protocol; heel-rise endurance and reactive-strength criteria before return to court.

THE FLOOR, TOGETHER · identical function, ten-fold re-rupture difference, zero difference in patient-reported score – who owns that decision, and does your answer change at day 6 versus day 21?

Why badminton, and a warning for chairs: in a 140-case badminton injury series Achilles rupture was the second commonest diagnosis (13.9%) – this is also the block that brings India's second sport into the programme and the only case landing in the 30–55 age band. Note: UKSTAR is not an operative-vs-non-operative trial – it randomised patients already on non-operative care between cast and functional brace. Correct any panellist who cites it as the op-vs-non-op evidence.

CB17 · Patellofemoral instability – when do you add bone?

The case: 17-year-old female state-level volleyball middle-blocker, third lateral patellar dislocation, this one non-contact on landing. Apprehension at 30°, positive J-sign; MRI shows Dejour C trochlear dysplasia, TT-TG 21mm, Caton-Deschamps 1.35, medial patellar facet chondral flap. Skeletally mature. **Surgeon 1** argues isolated MPFL reconstruction – the bony numbers are borderline, and MPFL-R alone returns 92.8% to sport with 1.9% recurrence; **Surgeon 2** argues MPFL-R *plus* tibial tubercle osteotomy – TT-TG 21 with patella alta is exactly the risk profile the definitive RCT excluded; **Surgeon 3** argues trochleoplasty – the only procedure correcting the primary pathology (pooled redislocation 0.35% across 927 patients), but a demanding operation on an already cartilage-injured knee. **Physiotherapist:** hip abductor and external-rotator loading rather than “VMO activation,” brace weaning, kinesiophobia, criterion-based staging. **S&C coach:** frontal-plane landing control in a jumping athlete, eccentric quadriceps capacity, growth/maturation load management in the female athlete.

THE FLOOR, TOGETHER · a 1mm difference in a measurement with known inter-observer variability decides whether this girl gets an osteotomy – show me your threshold, and your evidence for it.

Framing note for the chair: do NOT build this as MPFL reconstruction versus rehabilitation – that question is closing (3-year RCT: 16.7% vs 53.6% persistent instability). The live question is when to add bone, and it is live precisely because that same trial excluded Dejour D dysplasia and TT-TG >20mm – the patient this vignette describes.

CB18 · Multiligament knee injury / knee dislocation

The case: 24-year-old state-level kabaddi raider, ankle-hold by two defenders during a raid, forced hyperextension with valgus, unable to weight-bear; reduced on the mat, presents day 3 at a district hospital, referred day 6. Lachman 3+, posterior sag with 3+ posterior drawer, valgus laxity in full extension, dial test negative — **Schenck KD-IIIM**. MRI: ACL and PCL mid-substance ruptures, superficial MCL proximal peel-off, lateral meniscus posterior-root avulsion. **Neurovascular status is deliberately designed:** pulses palpable, ABI 1.0, but a dense common peroneal palsy with foot drop — including the nerve while excluding arterial injury is what keeps three positions live, since a vascular injury collapses the case into one algorithm. **Surgeon 1** reconstructs everything in one stage this week — reconstruction beats repair on arthrofibrosis (5.4% vs 11.6%) and failure (2.9% vs 5.7%); **Surgeon 2** braces/ex-fixes now and reconstructs at 3–6 weeks — no proven superiority of early over delayed, early surgery raises stiffness risk, and this is what most surgeons in resource-constrained settings actually do (argued as a deliberate choice, not a compromise); **Surgeon 3** repairs the MCL peel-off with internal-brace augmentation and owns the nerve — must defend the brace against an Indian systematic review finding it "as effective as not using it," and is the only one addressing the foot drop (AFO and observe vs early neurolysis vs planned tendon transfer). **Physiotherapist:** the stiffness-versus-laxity trap, which is genuine — push motion and you fight arthrofibrosis, protect the knee and the PCL graft creeps into posterior sag. **S&C coach:** return to performance where Tegner falls 7.12 to 4.59, rebuilding hyperextension-tolerant landing without a dynamometer or force plate.

THE FLOOR, TOGETHER · pulses are normal, the foot is dropped, there's no CT angiogram and no PCL allograft in the building — do you fix this knee this week, or splint it and send him 400km?

Overlap control with CB2 is mandatory — same sport, same joint. Separate by pathway, not pathology: CB2 is elective, late-presenting, single-ligament, rotational-laxity (graft choice and LET); CB18 is acute, bicruciate, limb-and-nerve, resource-constrained. The chair should rule "hamstring vs quad vs BTB" and "LET" out of order in CB18, and "knee dislocation" out of order in CB2. Do not schedule them adjacent. **Floor coda (Sep 2026):** isolated PCL was researched and rejected as its own block — ask "if this PCL tear were isolated, would you reconstruct it at all?"

Sources: Achilles three-arm RCT (Myhrvold et al., NEJM 2022) · operative vs non-operative meta-analysis (Ochen et al., BMJ 2019) · MIS/open/non-op meta-analysis (Attia et al., AJSM 2021) · accelerated vs delayed Achilles rehab (Coopmans et al., J Foot Ankle Surg 2022) · badminton injury series (Saragaglia et al., Eur J Orthop Surg Traumatol 2023) · MPFL-R vs rehab 3-year RCT (Straume-Naesheim et al., AJSM 2026) · isolated MPFL-R outcomes (Platt et al., AJSM 2021) · patella alta / tubercle risk (McGinley et al., AJSM 2025) · trochleoplasty pooled outcomes (Eikani et al., KSSTA 2024) · MLKI repair vs reconstruction (Floyd et al., OJSM 2025) · single-stage in irreducible dislocation (Sun et al., OJSM 2026) · internal bracing in MLKI (Randall et al., Indian J Orthop 2024) · MLKI outcomes and Tegner decline (D'Ambrosi et al., Indian J Orthop 2024) · kabaddi bicruciate case (Kembhavi et al., J Orthop Case Rep 2026). Full citations with DOIs in the committee's internal research notes.

RESERVE BLOCKS – CB15, CB19, CB20

Added September 2026 after benchmarking against Arthrex's "Back in the Game" meeting (June 2026), which exposed gaps in foot & ankle, cartilage, AC joint and failed surgery. Every block was held to the programme's standing checks: the sport must cause the injury by its real mechanism, the age must fit the epidemiology, every procedure must treat the stated pathology, and the debate must be genuinely unsettled. Turf toe, isolated PCL and pectoralis major were researched and rejected as blocks; they appear as floor-debate codas in CB20, CB18 and CB14.

CB15 · Revision & failed surgery – the failed ACL reconstruction

The case: 21-year-old male state-level football midfielder. Index surgery at 19: 8mm hamstring graft plus partial medial meniscectomy; back to matches at 7 months. Non-contact cutting injury in a match two years later. Lachman 2B, pivot shift grade 3; dial test negative, PCL and collaterals intact, neutral alignment. CT: femoral tunnel 12mm, high and shallow (a new anatomic tunnel would breach its back wall); tibial tunnel widened to 14mm; posterior tibial slope 13°; medial meniscus posterior horn deficient. The failure is multifactorial on purpose. *Settled, not debated:* revise with autograft (allograft re-ruptured more in MARS, and is scarce in India) – all three surgeons use BTB autograft.

Surgeon 1 argues one-stage anatomic revision plus LET, leaving the slope – in patients 25 or under LET was associated with fewer revisions regardless of slope; **Surgeon 2** argues one-stage slope-reducing osteotomy plus revision, no LET – slope $\geq 12^\circ$ was the strongest 20-year predictor of repeat injury, though a 46-expert Delphi could not agree a threshold and hardware reoperation ran at 26%; **Surgeon 3** argues two stages – graft both tunnels, revise at 4–6 months: lower failure (OR 0.12) but worse KOOS and more anterior knee pain. **Physiotherapist:** fear of reinjury is the commonest reason revision patients don't return (28%); track ACL-RSI from before surgery. **S&C coach:** no return before 9 months; graded match minutes.

THE FLOOR, TOGETHER · if 46 experts couldn't agree a slope threshold, is 13° a reason to cut bone in a 21-year-old at his first revision – or does LET do it more cheaply?

Overlap control with CB2: graft choice is fixed (BTB for all) and ruled out of order. The 12mm / 14mm / 13° values are template figures inside the argued ranges – replace with a real patient's before presenting.

CB19 · The syndesmosis that might be unstable

The case: 23-year-old male state-league footballer; tackle with the foot planted, forcing dorsiflexion and external rotation. Squeeze and external-rotation stress tests positive, anterior drawer negative. Weight-bearing radiographs: no fracture or frank widening, medial clear space at the upper limit of normal. MRI: AITFL torn, interosseous ligament partial, PITFL intact, superficial deltoid partial, deep deltoid in continuity, ATFL intact. No weight-bearing CT. Borderline because clinical tests cannot separate stable from unstable, and a deltoid injury plus partial syndesmotic tear is unstable in cadavers. *Settled, not debated:* stable injuries do well without surgery; suture-button beats screw (meta-analysis of 16 RCTs) – so the fixation device is not the debate. **Surgeon 1** argues "don't operate on a scan" – boot and criteria-based rehab; return to sport ~98% for both non-operative and suture-button care in elite athletes; **Surgeon 2** argues arthroscopy is the test – fix only if fibular translation is ≥ 2 mm, leave the deltoid; **Surgeon 3** argues rebuild it – suture-button plus AITFL augmentation plus deltoid repair, because only combined repair restored rotational stability (cadaveric evidence, which the floor should press). **Physiotherapist:** the dorsiflexion lunge as the tracking test; caution with early external-rotation loading after deltoid repair. **S&C coach:** straight-line, then cutting, contact last; watch for fibular pain after suture-button fixation.

THE FLOOR, TOGETHER · if non-operative and suture-button groups return at the same rate, what justifies rebuilding a deltoid that isn't fully torn?

Why football, and India: football is the only India-played sport with published syndesmosis evidence; none was found for cricket, kabaddi, hockey or badminton.

Overlap control: external-rotation mechanism, ATFL intact – CB13's lateral-ligament and CB6's chronic-instability debates are out of order.

CB20 · The subtle Lisfranc: fix, fuse or flex?

The case: 21-year-old female state-level basketball player; landed from a rebound on an opponent's foot. Plantar midfoot bruising, painful piano-key test at the second ray. Non-weight-bearing radiographs normal; standing bilateral films show the first cuneiform-second metatarsal gap 2.5mm wider than the other side; MRI shows a torn plantar Lisfranc ligament and injured intercuneiform ligament, no fracture. *Settled, not debated:* stable undisplaced injuries do well without surgery; residual displacement over 2mm predicts worse careers. **Surgeon 1** argues rigid fixation with planned hardware removal — the established athlete pathway, return at ~7.5 months, and a 10-year trial found bridge plating held up against fusion; **Surgeon 2** argues primary partial fusion — the only RCT in purely ligamentous injuries favoured fusion and it avoids a second operation, but no trial studied athletes; **Surgeon 3** argues flexible suture-button fixation — elite athletes back at 12–16 weeks, but case-series evidence only and the early advantage fades by one year. The JBJS 2026 commentary is titled “Fix or Fuse: The Lisfranc Debate Persists”. **Physiotherapist:** weight-bearing timelines differ completely by pathway; calf and intrinsic foot capacity, hop symmetry. **S&C coach:** landing and cutting mechanics, footwear, rebuilding jump volume.

THE FLOOR, TOGETHER · with no trial in athletes, a fusion that removes the second operation — or a motion-preserving repair that removes the certainty?

Turf toe coda (rejected as its own block — under 2% need surgery): “which forefoot and midfoot injuries must have weight-bearing or stress imaging before a return-to-play decision?” **Overlap control:** CB6 is also basketball — the female patient and midfoot pathology keep them distinct, but don't schedule them adjacent.

Sources: MARS Group, AJSM 2021 · van der Wal et al., AJSM 2026 · Gopinath et al., AJSM 2026 · Salmon et al., AJSM 2018 · Wackerle et al., J ISAKOS 2025 · Ifarraguerri et al., Knee Surg Relat Res 2025 · McPherson et al., AJSM 2019 · Grindem et al., BJSM 2016 · Mauntel et al., AJSM 2017 · Lubberts et al., BJSM 2019 and Foot Ankle Int 2019 · Calder et al., Arthroscopy 2016 · Cehic et al., JBI Evid Synth 2026 · Li et al., J Orthop Surg Res 2025 · D'Hooghe et al., BJSM 2020 · Bachmaier et al., AJSM 2025 · Porter et al., Foot Ankle Int 2018 · McHale et al., AJSM 2017 · Grün et al., JBJS 2026 · Ly & Coetzee, JBJS 2006 · Ponkilainen et al., Foot Ankle Int 2024 · Sullivan et al., OJSM 2022 · Gupta et al., Curr Rev Musculoskelet Med 2023. DOIs in the planning document, section 3.

RESERVE BLOCKS – CB21, CB22

CB21 · The focal cartilage defect in the young footballer

The case: 24-year-old male semi-professional footballer; non-contact twist 8 months ago, knee swollen but stable; medial pain and effusion after every match since; 4 months of supervised rehab failed. MRI: $18 \times 14\text{mm}$ ($\sim 2.5\text{cm}^2$) full-thickness defect of the central weight-bearing medial femoral condyle, well-shouldered, subchondral plate intact; ACL and both menisci intact. Neutral alignment, so osteotomy is not indicated. At 2.5cm^2 all three positions are legitimate – a clash of schools, not a size triage. *Settled, not debated:* plain microfracture is effectively obsolete for footballers per the 2025–26 FIFA/ICRS/Aspetar consensus, so nobody defends it. **Surgeon 1** argues single-stage osteochondral autograft transfer – hyaline cartilage and the fastest return (93% back to prior level vs 52% after microfracture in athletes), but 2.5cm^2 needs several plugs; **Surgeon 2** argues two-stage cell-based cartilage repair – better 10-year function in an RCT, favoured for players prioritising the long term, but two operations and the cost; **Surgeon 3** argues single-stage marrow stimulation under a cell-free scaffold – the affordability case, on low-certainty evidence, and must concede the consensus rating. **Physiotherapist:** each construct loads differently; effusion-guided, criteria-based progression. **S&C coach:** graded return to high-speed running, deceleration and impact.

THE FLOOR, TOGETHER · does a scaffold-augmented marrow stimulation today burn the bridge to cell-based repair tomorrow?

India reality check – corrects an earlier assumption: cell-based cartilage repair *is* available in India (regulator-authorised, in use since 2008) – cost, not availability, is the barrier. Fresh osteochondral allograft is not a realistic Indian option and is excluded. **Overlap control:** not CB4 (56, varus, degenerative) or CB10 (adolescent OCD). Use generic names on slides.

CB22 · Grade III AC joint dislocation: IIIA or IIIB?

The case: 24-year-old male state-level road cyclist; crashed onto the point of the shoulder; seen at day 10. Prominent reducible distal clavicle, tender AC and coracoclavicular ligaments, pain on cross-body adduction. Zanca views: coracoclavicular distance $\sim 80\%$ greater than the other side (grade III); axillary view excludes grade IV; the cross-body view shows the clavicle overriding the acromion – the radiographic sign of IIIB, though at day 10 nobody can say whether the scapular dysfunction will settle. **Surgeon 1** argues non-operative first – a meta-analysis of five RCTs found no Constant-score difference at 2 years, and surgery added minor complications; **Surgeon 2** argues early arthroscopic coracoclavicular stabilisation – the “no difference” result is statistically fragile and came from trials averaging age 39, while a younger RCT favoured surgery on UCLA score and cosmesis, and return to play reaches 98.7%; **Surgeon 3** argues two-plane stabilisation – an overriding clavicle is horizontal instability that vertical-only fixation leaves untreated, though its RCT showed no clinical difference at a year. **Physiotherapist:** what rehab must show, and by when, before calling it IIIA or IIIB. **S&C coach:** handlebar and aero-position loading, early indoor riding, re-crash risk.

THE FLOOR, TOGETHER · the meta-analyses pool trials of 39-year-olds – is a trial of rehab evidence-based for this 24-year-old, or does it waste the window for fixation?

Why cycling: 51 of 106 AC dislocations in a Danish cohort came from cycling, peaking at age 20–24 (evidence is Danish and American, not Indian). **Note:** this is the fifth shoulder-region block – if a shoulder reserve is promoted, CB22 fits an Indian audience better than CB12's baseball pitcher. Raise low-cost hook-plate fixation from the floor.

Sources: Andrade et al., Arthroscopy 2016 · Kon et al., BJSM 2025 · Papakostas et al., BJSM 2026 · Gudas et al., Arthroscopy 2005 and AJSM 2012 · Kunze et al., AJSM 2025 · Snow et al., Osteoarthritis Cartilage 2026 · Reddy et al., BMJ Open Sport Exerc Med 2026 · Minas et al., AJSM 2009 · Pardiwala et al., Knee 2024 · Haugaard et al., Shoulder Elbow 2022 · Beitzel et al., Arthroscopy 2014 · Lameire et al., OJSM 2026 · Althoff et al., Orthopedics 2026 · Lara et al., AJSM 2026 · Cleary et al., AJSM 2023 · Ambrožič et al., J Clin Med 2025 · Lim et al., JSES Rev Rep Tech 2025. DOIs in the planning document, section 3.

YOUNG SURGEON FORUM FINAL – CASE VS CASE KNOCKOUT

Adapted from “Back in the Game”, but built to keep every promise already published on the abstracts page: 8+2 min podium free papers, e-posters, the Young Surgeon Forum and its awards. **Slot:** Day 2, Hall B, 15:42–17:00, replacing rapid-fire free papers; all podium papers move to Day 1, 18:17, both halls (~14 talks); beyond that, e-poster. No society meeting researched runs an audience-voted knockout — their awards are jury-selected — so **judges decide (80%) and the audience vote is capped at 20%**. **Entry:** PG students (Head of Department letter) or young consultants (definition pending); 8 finalists chosen by blinded review of case submissions, seeded so no two from the same unit meet in round one; de-identified cases with written patient consent.

TIME	STAGE	PER MATCH
15:42	Rules, judges' disclosures, voting explained	—
15:45	Quarter-finals, 4 × 10 min	Case A 3' · case B 3' · judges question both 3' · score and vote 1'
16:25	Semi-finals, 2 × 9 min	Each shows their own case's outcome or complication and what they'd change (2.5' each) · questions 3' · score 1'
16:43	Final, 12 min	Unseen case from the committee: reveal 1' · three staged questions, 1.5' per finalist · score 2'
16:55	Tally, hall reset	Winner announced at 17:30 Awards & valedictory

Rubric (3 judges, 100 points): clinical reasoning 30 · use of evidence 20 · rehab & return-to-sport plan 15 · honesty about outcomes and complications 15 · clarity 10 · handling questions 10. **Vote safeguards:** one vote per registration badge; voting opens only after both cases; judges lock scores first; under 30 votes, judges count 100%; judges recuse for their own unit. **Prize** paid from organiser funds, never an industry-funded cash prize.

INDUSTRY INVOLVEMENT & CPD INDEPENDENCE

Corrects a planning assumption: no published Telangana Medical Council rule says product-branded sessions break CPD accreditation. What Indian rules actually require: NMC's 2023 conduct regulations are in abeyance, so the **IMC Regulations 2002, clause 6.8** apply — to individual doctors: no industry gifts, travel, hospitality or cash; affiliations disclosed; and a doctor *“shall not endorse any drug or product of the industry publically”*. TGMC's CPD requirements are procedural (observer, an ethics/GCP session, consent for workshop patients, consider free registration for doctors over 65). The device-industry code (UCMPMD 2024) permits booth demonstrations. NMC is reported to be tightening rules on sponsored CME — get TGMC's position in writing.

Required by Indian rules

No faculty member publicly endorses a product — a podium “technique highlight” of a named system risks clause 6.8 · no industry-paid travel, hospitality, gifts or cash to doctors, including prizes · faculty industry relationships disclosed · TGMC observer, ethics/GCP plenary (Day 1, 17:32 — already in the grid), patient consent for workshops.

Best practice (ACCME / EACCME — not binding in India)

Generic names only in titles and slides; no logos in either hall · a disclosure slide opening every talk · live demos only in the exhibition area during breaks, labelled “Industry — not part of the CPD programme” and excluded from CPD hours · sponsors acknowledged on the website and an acknowledgements page, never per-session branding · a written sponsor agreement giving sponsors no say on content or faculty.

Committee decisions this creates

Podium capacity drops to ~14 talks · define “young consultant” before abstracts open · decide over-65 registration (the registration page has none) · confirm the TGMC fee (council minutes of Feb 2024 record Rs 50,000 for a national conference; earlier notes carried Rs 10,000) · rank the 13 reserve blocks before outreach on any of them. Full detail: planning document, sections 7, 11 and 12.

REHABILITATION, PHYSIOTHERAPY & STRENGTH AND CONDITIONING

Format: 12–15 minute talks; blocks close with a moderated discussion rather than per-talk Q&A. Two ticketed, capped hands-on workshops — these are the reason physiotherapists pay to attend rather than watch online. **Recruit S&C faculty, not only physiotherapists:** an all-physio track loses the S&C audience by mid-morning of Day 1. Sports-science and nutrition content lives in this hall — see the two dedicated blocks below.

B1 · Criteria-based rehabilitation & the RTP decision

"Time is not a criterion" — what criteria-based actually means when you write the discharge note · what proportion of your athletes would truly pass a full test battery at 9 months post-ACLR · return to participation → sport → performance · psychological readiness (ACL-RSI) and the athlete whose knee passes but whose head doesn't · who owns the decision, and how it is documented. Closes by building a one-page RTP criteria sheet live from the floor.

DEBATE · No athlete should be cleared for pivoting sport before 9 months post-ACLR

B2 · Objective testing, screening & technology

Force plates in a 400-sq-ft clinic: the five countermovement-jump metrics worth reporting and the twenty that aren't · handheld dynamometry done properly — belt fixation, normalisation, minimum detectable change · isokinetics vs field testing in 2027 · movement screening after FMS: what survived the validity literature · markerless motion capture — good enough to change a clinical decision? · **panel:** "What would you buy first with ₹5 lakh?"

B3 · Tendinopathy & load-driven pain

Tendon pain as a loading problem, in one slide · isometrics for analgesia: where "45 seconds × 5" came from and how often it fails · heavy slow resistance vs eccentrics for patellar tendinopathy — the network meta-analysis verdict · insertional vs mid-portion Achilles: why the same programme fails one · gluteal tendinopathy — compression is the enemy, stretching is the trap · energy storage and release, the phase everyone skips.

W1 · Hands-on carousel TICKETED · CAPPED ~30/STATION

Station 1 — Objective testing bench: live CMJ and drop jump on volunteers, reading the force-time curve on screen; dynamometry with vs without belt fixation, showing the tester-strength artefact; writing the actual report line a surgeon and a coach will each read.

Station 2 — Tendon loading lab: Spanish squat rigged with a belt and pole, no proprietary kit; HSR tempo work; modifying every exercise for a clinic with one bench, one step and one barbell; the 24-hour pain-response rule on case vignettes.

Station 3 — Plyometric progression: pogo → hop → bound ladder with landing-quality criteria; take a 3-month post-ACLR athlete to month 7 using only cones, a box and floor space.

W2 · Field-based RTP testing & hamstring reconditioning HANDS-ON

Build a 40-minute RTP battery with no lab — 505 change of direction, hop series with a tape measure, Askling H-test, sprint exposure with timing gates, and honest caveats on phone-app timing · hamstring L-protocol and Nordic technique coached hands-on, with regressions · sprint reintroduction dosing and the criterion for the first max-effort sprint · the pass/fail sheet you hand the coach.

B5 · Muscle injury & the hamstring

Classifying muscle injury (BAMIC, MLG-R) — and whether the MRI changes what you do on Monday · L-protocol vs conventional strengthening: 28 days vs 51 days, and what newer reviews say about reinjury · lengthened-state training and the Nordic: fascicle length vs adherence reality · calf and adductor — the two you are under-rehabbing · from return to running to return to sprinting.

B6 · Load management & injury risk

What happened to ACWR — from "predicts injury" to a cluster RCT in 482 elite youth footballers showing no difference in injury rates · metrics that survive a real Indian club season: sRPE, weekly change, chronic capacity, low-tech monitoring that actually gets collected · bowling workload and the fast bowler's back · sleep, energy availability and travel — the load that never appears in the GPS file.

B7 · The athlete who isn't a 25-year-old male

Youth resistance training is protective — the position-statement evidence, and how to say it to a parent and a coach · the maturing athlete: apophyseal injury, load modification not rest · the female athlete and the menstrual cycle: what genuinely low-quality evidence permits you to claim · REDs as a modifiable rehabilitation variable · para-athlete rehabilitation and why your normative data doesn't apply.

B4 · Adjuncts under scrutiny RESERVE

Blood flow restriction: what 2024–25 meta-analyses actually show (comparable, not superior) and the three groups where the cuff earns its place · manual therapy as a short-half-life adjunct — using it without building dependence · shockwave, dry needling, and the physiotherapist's filter for injectable-adjacent claims.

Fuelling the return — nutrition, energy availability & recovery DAY 2

Nutrition for injury recovery: protein targets and the leucine threshold on a vegetarian Indian diet · energy availability and REDs as a modifiable rehabilitation variable — screening without a DXA · hydration, sweat sodium and heat in Indian training conditions · the five supplements that survive scrutiny, plus contamination risk and strict liability · sleep and recovery as the load that never appears in the GPS file.

The mind of the returning athlete DAY 2

Psychological readiness: ACL-RSI and I-PRRS as gate criteria, not paperwork · fear-avoidance — identifying the athlete who is physically cleared but will not compete · the conversation that resets an athlete's expectations · disordered eating in aesthetic and weight-category sport: what the physiotherapist, the coach and the dietitian each own.

Sources: RTS tests and criteria after ACLR scoping review 2025 · AOSSM RTP metrics (Winter 2025) · LSI overestimates knee function (JOSPT) · BFR vs general rehab after ACLR (KSSTA 2025) · eccentric vs isometric vs HSR network meta-analysis · hamstring rehab comparative effectiveness · ACWR systematic review and meta-analysis · FMS predictive validity · NSCA youth resistance training position statement · IFSP / World Physiotherapy Congress 2025 · kabaddi knee injury epidemiology.

THE THREADS THAT MAKE IT OURS

A translated Western programme is the failure mode of a new conference. These threads run through both halls and give HSC 2027 an identity no international meeting can copy.

- **Kabaddi.** Indian series data show ACL tears in 89.5% of injured players, 68.4% with meniscal involvement, **57.9% never returning to the sport**, and 16.6 months mean time lost among those who do. That single dataset earns a case in the ACL, meniscus, hip/groin and rehabilitation sessions.
- **Cricket bowling workload** and the fast bowler's back — across surgical, rehabilitation and load-management halls.
- **Heat and humidity.** Heat policy, sweat sodium, acclimatisation. Non-negotiable for an Indian audience and absent from most imported programmes.
- **Vegetarian athlete nutrition** — protein targets and the leucine threshold on Indian diets.
- **Doping.** India has led WADA's violation table three years running. A reserve block on the 2026 Prohibited List for team clinicians is the most valuable single hour the conclave could add.
- **Rehabilitation without a lab.** Most Indian clinics have no isokinetic dynamometer or force plate. Every rehab talk should state what to do without one.
- **Late presentation.** Kabaddi ACL injuries present at a mean of ~14 months. Programme it as a clinical reality, not a footnote.

WHAT WE NEED SIGNED OFF

1 · Two days, or two plus a pre-conference day?

The research yields roughly three days of strong content. A 29 January pre-conference day would hold the hands-on workshops, any dry-lab work and the reserve blocks. **Take this decision first — it determines the venue contract.**

2 · Confirm the two-hall split, the hall titles, and the Hall A case-based format.

Hall A doubles as the plenary hall and must seat the full delegate count. Sports science and nutrition sit inside Hall B rather than in a hall of their own. Hall A's nine case blocks — one real patient, three surgeons, physiotherapist, S&C coach, floor debate — are the Chairman's own design and the meeting's actual point of difference; confirm the committee is aligned on running one deep case per hour rather than a survey of sub-topics before faculty outreach goes further.

3 · Faculty scale — Hall A specifically.

Hall A's 9 case blocks need 3 surgeons + 1 physiotherapist + 1 S&C coach each — 45 seats, fewer with deliberate reuse of a strong confirmed name across two blocks. Plus Hall B's own 55–70 lecture slots. Real, sourced candidates exist for every Hall A block; CB1 (the flagship shoulder case) is the best-staffed and should be confirmed first — a confirmed CB1 surgeon becomes the strongest recruiting tool for the rest. One seat remains genuinely open after two research passes: a surgeon or sports physician for CB8 who speaks specifically to fast-bowler lumbar stress fracture — this needs a personal introduction through the committee's own network, not further search. Full candidate list, with sources, in the committee's internal outreach document.

4 · Which debates run in Hall B.

Hall A's nine blocks are already debate by design. In Hall B, two debate-format lectures per day is the ceiling — more and they stop feeling like events.

5 • The ethics / GCP session stays as a plenary.

TSMC's Declaration Proforma requires it for CPD accreditation. Non-negotiable.

6 • Workshop ticketing.

Hands-on sessions must be capped and ticketed at registration with per-station rosters. Uncapped "hands-on" becomes a lecture with mats on the floor.

7 • Equipment sponsorship boundary.

Force plates and dynamometers will be vendor-supplied — tie them to an exhibitor package, but keep teaching faculty independent of the vendor and say so from the podium. This is the single biggest credibility risk in the rehabilitation track.

8 • Free-paper prizes per track.

So physiotherapy and nutrition entries don't compete against surgical case series.

Two research caveats, stated plainly. The ISAKOS, AOSSM and ESSKA programme pages blocked direct retrieval, so Hall A structure is reconstructed from search-indexed programme and abstract listings rather than read off the printed programme — worth checking the ESSKA 2026 highlight-lecture list before finalising. And IASMCN does not publish a session-level programme in machine-readable form, so the sports-science content now folded into Hall B is built from the consensus literature and Indian data rather than a competitor programme.